

Medical Information Chatbot Report

Question: what are the symptoms of this disease

Retrieved Context:

o check therapeutic response, compliance, and repeat spirometry.

----- ELECTRONICALLY SIGNED BY: Dr.
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----- ring deep inspiration. - ENT: Boggy, pale nasal turbinates consistent with active allergic rhinitis. No sinus tenderness. - Cardiovascular: Regular rhythm, normal S1/S2 without gallops or rubs. DIAGNOSTIC TESTING (SPIROMETRY & PULMONARY FUNCTION): - Pre-Bronchodilator FEV1: 2.15 L (62% of predicted value) - Pre-Bronchodilator FEV1/FVC Ratio: 64% (Indicative of obstructive defect) - Post-Bronchodilator FEV1: 2.58 L (An increase of 20% and +430mL in volume) - Clinical Interpretation: Highly significant and positive bronchodilator reversibility, confirming hyper-reactive airway disease. ASSESSMENT & DIAGNOSIS: 1. Moderate Persistent Asthma (ICD-10: J45.40) - Uncontrolled on current lifestyle. 2. Chronic Allergic Rhinitis (ICD-10: J30.9) - Exacerbating airway reactivity. MANAGEMENT & ACTION PLAN: 1. Maintenance Pharmacotherapy: Initiate Fluticasone propionate/Salmeterol (Advair Diskus 250/50 mcg), 1 inhalation orally twice daily. Emphasize strict compliance regardless of active symptoms. 2. Rescue Pharmacotherapy: Prescribe Albuterol Sulfate HFA inhaler, 90 mcg, 2 puffs orally every 4-6 hours as needed for acute bronchospasm/wheezing. 3. Allergy Control: Initiate Cetirizine 10 mg orally once daily at bedtime and Fluticasone propionate nasal spray, 2 sprays in each nostril daily. 4. Patient Education: Provided written Asthma Action Plan. Educated on peak flow monitoring and trigger avoidance (dust mites, industrial particulates). 5. Follow-up: Re-evaluate in clinic in 4 weeks t...

Generated Answer:

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